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The Complete Claim System

The Health Insurance Claim Playbook

India Edition 2026-27

How to get your health insurance claim approved - or overturned

Understand your policy. Know your rights. File the claim right. Fight back if you are short-changed.

Every rule verified from IRDAI, the Income-tax Act 2025 and the National Health Authority

About this playbook

This is a general guide to how health insurance claims work in India and how the rules protect policyholders. It is written for individuals and families who already hold, or are about to buy, a health insurance policy.

It is not legal, financial or insurance advice for your particular policy or dispute. Policy wordings differ between insurers and products, and the rules described here can change. For a live claim or a dispute above Rs. 50 lakh, take professional advice, and always confirm the current position on the IRDAI website or with your insurer.

The figures and rules in this edition were verified against official sources current to October 2026. The main sources are listed at the end of the guide.

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Contents

About this playbook	2
Contents	3
Introduction	7
The claim is the product	7
What this playbook does	7
Part 1. Why claims shrink - the eight levers	8
The claim you file is not the claim you get	8
Lever 1: The sum insured - the wall you hit	8
Lever 2: Room-rent caps and proportionate deduction	8
Lever 3: Co-payment	8
Lever 4: Sub-limits	9
Lever 5: Deductibles and top-up policies	9
Lever 6: Waiting periods	9
Lever 7: Non-medical items and consumables	9
Lever 8: The disclosure clause	9
Anatomy of a real claim	10
What the numbers say	10
Part 2. Read your policy in thirty minutes	12
Start with the Customer Information Sheet	12
The standard product: Arogya Sanjeevani	12
The twelve questions to answer before you ever claim	12
Part 3. The IRDAI rulebook that is on your side	13
The one-hour and three-hour clock	13
The fifteen-day reimbursement rule - and the interest you are owed	13
No rejection without a committee	13
The sixty-month moratorium: your claim becomes incontestable	13
The thirty-six-month cap on pre-existing disease	14
Cashless Everywhere	14
NHCX: the claims highway being built	14

No GST on your individual health premium	14
Free look, grace period, portability, migration	15
Part 4. The claim, step by step	16
Planned hospitalisation: the two-week playbook	16
Emergency admission: the first forty-eight hours	16
The sixty-minute cashless script	16
The document vault: the master checklist	17
The discharge-day script	17
Reimbursement: the complete playbook	17
Part 5. When they say no - the escalation ladder	19
Decode the rejection letter before you argue	19
Rung 1: the Grievance Redressal Officer	19
Rung 2: Bima Bharosa	19
Rung 3: the Insurance Ombudsman	19
Rung 4: consumer court	20
The three letters that win	20
The medical-necessity letter: what to ask your doctor for	21
Part 6. Special situations	22
Maternity and newborn claims	22
Claims for parents and senior citizens	22
Top-up and super top-up claims	22
Critical illness and fixed-benefit plans	22
Day-care procedures	22
Accidents, medico-legal cases and FIRs	23
Part 7. Ayushman Bharat and government cover	24
The world's largest health assurance scheme	24
The seventy-plus expansion	24
How to get the Ayushman card	24
Part 8. The tax angle: Section 126	25
Your premium still cuts your tax bill	25
What you can claim	25

The two conditions that matter	25
--	----

Part 9. Buy better to claim better **26**

The five features that decide your payout	26
How to read a claim settlement ratio	26
Ten questions to ask before you buy	26

Part 10. Your one-page cheat sheets **27**

Emergency card - keep this in your wallet	27
Document vault - photograph before you hand over	27
Escalation ladder - in order	27
The four numbers to remember	27

Questions people ask **28**

Does a cashless rejection mean my claim is denied?	28
Can an insurer reject my claim just because I filed it late?	28
The insurer asked for a document the hospital already sent. Must I provide it?	28
How long do I have to file a complaint with the Ombudsman?	28
Is the Ombudsman really free, and do I need a lawyer?	28
My policy lapsed for a month. Does it matter?	28
What is the difference between a rejection and a repudiation?	28
Will the insurer pay for the room I actually stayed in?	28
Are consumables and 'non-medical' items ever payable?	29
Does the 60-month moratorium apply to my old policy?	29
What is Cashless Everywhere, and can I use it at any hospital?	29
Has GST really gone from my health premium?	29
How much cover should I actually buy?	29
Can my senior parents get cover if they already have a condition?	29
Do I need to keep original bills if I claimed cashless?	29
What if the insurer pays less than I expected?	29
Is it worth hiring a lawyer for a claim dispute?	29
Where do I check my insurer's official network and grievance contacts?	30
What if I have two health policies?	30
Does my policy cover expenses before and after hospitalisation?	30
Can the insurer raise my premium at renewal?	30
What is a restoration benefit, and do I need it?	30

Glossary of terms	31
Sources	33

Introduction

The claim is the product

You did not buy a health insurance policy. You bought a promise that someone else will pay the hospital bill. That promise is only tested on the day you are admitted - and that is the day you have the least time, the most stress, and no idea what the rules say. This playbook exists for that day.

Here is the uncomfortable truth the brochures hide. In 2024-25, Indian health insurers processed about 3.26 crore claims and paid out roughly Rs. 94,248 crore. The claim-paid ratio by number of claims was 87.5% - which sounds reassuring until you notice that roughly one in eight claims was not fully paid. Some were repudiated outright. Far more were 'disallowed' in part: the insurer paid something, but quietly trimmed the bill using a room-rent cap, a co-payment, a sub-limit, or a list of 'non-medical' items. That trimming is where most of the money leaks, and most of it is written into the policy you already own.

The good news is that the rules in India now sit firmly on the policyholder's side - far more than they did even five years ago. The regulator, IRDAI, has put hard clocks on insurers, banned silent rejections, cut the pre-existing disease waiting period, and created a free, no-lawyer route to overturn a bad decision. Very few people know these rules exist. This guide puts every one of them in your hands.

What this playbook does

It is written to be used, not just read. It is arranged in the order a real claim happens: understand the policy, know your rights, file the claim properly, and fight back if you are short-changed. Every fact is drawn from official sources - IRDAI circulars, the Income-tax Act 2025, PIB releases, the National Health Authority and the Insurance Ombudsman Rules - and the dates are current to the 2026-27 policy year.

You do not need to be a finance expert. You need to know eight things about your policy, four numbers that insurers must obey, one checklist of documents, and one escalation ladder. That is the whole game. Everything else in this guide is detail that makes those four things work.

A note on currency: all figures are in Indian rupees and written as 'Rs.' throughout. Amounts, deadlines and rules were verified against official sources current to October 2026. Insurance rules change; confirm the latest position on the IRDAI website or with your insurer before acting on a time-critical claim.

Part 1. Why claims shrink - the eight levers

The claim you file is not the claim you get

When a hospital sends a bill of Rs. 2,00,000, it is easy to assume the insurer pays Rs. 2,00,000 minus a small deduction. In practice, an insurer can reduce that bill through eight separate mechanisms, and several of them can apply to the same claim at once. Understanding them before you are admitted is the difference between a 95% payout and a 60% payout.

The official reasons for claim disallowance, as listed by IRDAI itself, are: exceeding the sum insured, the co-payment clause, sub-limits, the deductible in top-up policies, room-rent capping, proportionate charges, and non-medical expenses. Add the waiting-period rules and the disclosure clause and you have the complete map of where your money goes. Let us take them one at a time.

Lever 1: The sum insured - the wall you hit

The sum insured is the maximum the insurer will pay in a policy year. It is not a target; it is a ceiling. If your cover is Rs. 5 lakh and the bill is Rs. 7 lakh, the extra Rs. 2 lakh is yours. Most families buy too little cover. A single cardiac surgery with a week in ICU, or an oncology cycle, can exhaust Rs. 5 lakh quickly. IRDAI data shows the average claim paid in 2024-25 was only about Rs. 28,910, which tells you most claims are small - but the ones that hurt are the large ones, and those are exactly the ones that breach a thin sum insured.

The fix is simple and cheap: buy a base cover you can afford and stack a super top-up above it. A super top-up is a second policy that pays once your base cover is exhausted, and it costs a fraction of the base premium because you carry the first slice yourself.

Lever 2: Room-rent caps and proportionate deduction

Many policies cap the room you may occupy - commonly at 1% or 2% of the sum insured per day, or a flat figure like Rs. 5,000. The trap is not the room charge itself. It is the 'proportionate deduction' rule. If you choose a room that costs more than your cap, the insurer does not just trim the room rent. It trims most of the rest of the bill in the same proportion.

Here is how it bites. Suppose your cap is Rs. 5,000 a day but you take a Rs. 10,000 room. The insurer divides the allowed rate by the actual rate - 5,000 divided by 10,000 - and pays only half of almost everything else: surgeon fees, OT charges, nursing, diagnostics. A small comfort upgrade can halve your payout. The exception, in the standard policy wording, is the cost of medicines, consumables, implants, devices and diagnostics, which are usually protected from proportionate deduction.

The practical rule: if your policy has a room cap, take the room that fits it. Ask the hospital's insurance desk for the category your policy allows, and do not upgrade without checking what it costs you in deduction.

Lever 3: Co-payment

A co-payment is a fixed percentage of every admissible claim that you always pay. A 5% co-pay on a Rs. 2,00,000 bill is Rs. 10,000 out of your pocket, every time, no matter how large your cover. Some policies charge a higher co-pay for senior citizens - often 10% or 20% - which is why cover for parents can quietly cost far more than the sticker price suggests.

Co-pay is a permanent tax on your claim. When you compare policies, a plan with a 5% co-pay is not really offering the same cover as one with none. Add the co-pay to the premium when you judge value.

Lever 4: Sub-limits

A sub-limit caps a specific treatment, even if you have plenty of sum insured left. The standard Arogya Sanjeevani product, for example, pays only up to 25% of the sum insured or Rs. 40,000 (whichever is lower) for a cataract operation, and up to 50% of the sum insured for listed modern treatments. A modern, robotic or day-care procedure can therefore be trimmed hard even when your total cover is untouched.

Disease-wise sub-limits (for cardiac procedures, joint replacements, cataracts and so on) are among the most common causes of 'partial settlement'. Read the sub-limit table in your policy schedule before any planned procedure, and ask the hospital for a cost estimate that fits inside it.

Lever 5: Deductibles and top-up policies

A deductible is an amount you agree to pay before the policy responds. It is most common in top-up and super top-up plans: a super top-up with a Rs. 5 lakh deductible pays nothing until your total bills cross Rs. 5 lakh, and then pays above that. That is exactly why it is cheap - you are funding the first slice. It is not a trick, but it must be understood: a top-up is not a second base policy, and claiming on it for a small bill will get you a zero.

Lever 6: Waiting periods

Three waiting periods can stand between you and a payout. First, the initial waiting period - typically 30 days from the start of the policy, except for accidents. Second, disease-specific waiting periods - usually one to three years for listed conditions such as hernia, cataract, joint replacement or piles. Third, the pre-existing disease (PED) waiting period, which since 2024 can be no longer than 36 months.

A claim filed inside a waiting period is not fraud and not a moral failing. It is simply not covered yet. The single most powerful protection here is continuity: every time a policy lapses and is restarted, these clocks reset to zero. Never let a policy lapse.

Lever 7: Non-medical items and consumables

Indian hospital bills bundle in a long list of items that insurers class as 'non-medical': administrative charges, admission kits, gloves, syringes, masks, IV sets, sanitary items, television and telephone, attendant charges, and a range of consumables. These are routinely excluded or trimmed. The exact list varies by insurer, and the amount is often several thousand rupees on a short admission and much more on a long one.

You cannot usually make the insurer pay these. What you can do is know they exist, expect the deduction, and not treat it as an unfair rejection. Some insurers now offer riders or add-ons that cover consumables; if non-medical deductions annoy you, ask about them at renewal.

Lever 8: The disclosure clause

When you bought the policy, you answered a proposal form about your health history. If the insurer later finds that a material fact was not disclosed - a diagnosed condition, a medication, a past test - it can treat the policy as void and forfeit the premium, or repudiate the claim. This is the most feared lever because it can wipe out the entire claim, not just trim it.

Two things tame it. First, the 2024 rules cut the PED waiting period to 36 months and made a policy incontestable after 60 months of continuous coverage except for proven fraud. Second, courts have repeatedly held that an insurer must show the non-disclosure was material - that it actually mattered to the claim. An honest mistake, corrected or immaterial, is not a free pass for the insurer. Be scrupulous at the point of sale; the whole problem disappears.

Anatomy of a real claim

Abstract rules become obvious when you watch them work on a bill. The illustration below shows how a Rs. 2,10,000 hospital bill gets trimmed to a payable of about Rs. 1,33,000 - and, more usefully, shows which single decision caused most of the loss. The policy here is a typical Rs. 5 lakh cover with a room cap of Rs. 5,000 a day, a Rs. 10,000 ICU cap, a 5% co-pay, and no cover for non-medical items. The patient chose a Rs. 8,000-a-day room.

How a Rs. 2,10,000 bill becomes Rs. 1,33,000

Line item	Billed (Rs.)	Allowed (Rs.)	Why
Room rent, 4 days at 8,000	32,000	20,000	Capped at 5,000 a day
ICU, 1 day	15,000	10,000	Capped at 10,000 a day
Surgeon and anaesthetist	55,000	34,375	Proportionate deduction (5,000 / 8,000)
Operation theatre	20,000	12,500	Proportionate deduction
Nursing and procedures	18,000	11,250	Proportionate deduction
Diagnostics	22,000	22,000	Protected from proportionate deduction
Medicines	30,000	30,000	Protected from proportionate deduction
Non-medical items	18,000	0	Excluded by the policy
Total admissible	2,10,000	1,40,125	
Less 5% co-payment		7,006	Always borne by you
Net payable by insurer		1,33,119	About 63% of the bill

Illustrative figures. Read the last row slowly: the insurer paid about 63% of the bill, and the patient funded Rs. 76,881 from savings. The single largest avoidable loss was not the room rent itself - it was the proportionate deduction, roughly Rs. 46,000 across three lines, triggered by a Rs. 3,000-a-day room upgrade. Had the patient taken the Rs. 5,000 room the policy allowed, most of that money would have stayed in the payout.

This is the whole argument of Part 1 in one table. You cannot argue your way out of a room cap at the counter. You avoid it by choosing the right room and by understanding your policy before the admission, not after.

What the numbers say

IRDAI's own figures show the scale of the fight. The claim-paid ratio by number of claims was 85.66% in 2022-23, fell to 82.46% in 2023-24, and recovered to 87.50% in 2024-25. In 2024-25, about 58% of claims were settled cashlessly. In the same year, 1,37,361 general and health insurance grievances were filed on the Bima Bharosa portal, and 93% were disposed of within the year. Those grievances are people who refused to accept a shortfall. Many of them won.

Health insurance claims in India - the official picture

Measure	Figure
Health claims processed, FY 2024-25	3.26 crore
Amount paid, FY 2024-25	Rs. 94,248 crore
Average claim paid	Rs. 28,910
Claims settled cashlessly	About 58%
Claim-paid ratio by number (FY 2024-25)	87.50%
Claim-paid ratio (FY 2023-24)	82.46%
Grievances filed on Bima Bharosa (FY 2024-25)	1,37,361
Grievances disposed in-year	93%

Source: IRDAI Annual Report 2024-25 and PIB release dated 26 March 2026.

Part 2. Read your policy in thirty minutes

Start with the Customer Information Sheet

Since 2024, every health policy must come with a one-page Customer Information Sheet, or CIS. It is the single most useful document you own and almost nobody reads it. The CIS must state, in plain language: the type of insurance, the sum insured, the cover provided, a summary of exclusions, the sub-limits, the deductibles, the co-payment, the waiting periods, the free-look period, the renewal terms, the portability and migration rules, the moratorium period, and the grievance-redressal contacts including your Insurance Ombudsman.

If you cannot find your CIS, ask your insurer or your agent for it. Reading those fourteen lines will tell you more about what your policy will actually pay than any brochure. If a term in the CIS is unclear, that is the term that will bite you.

The standard product: Arogya Sanjeevani

IRDAI mandates a standard health product, Arogya Sanjeevani, that every general and health insurer must offer. Because its features are fixed by the regulator, it is a useful reference point for reading any other policy. Its known terms include a room-rent cap of 2% of sum insured or Rs. 5,000 a day, whichever is lower; an ICU cap of 5% or Rs. 10,000 a day; a cataract sub-limit of 25% of sum insured or Rs. 40,000, whichever is lower; listed modern treatments capped at 50% of sum insured; a mandatory 5% co-payment on every admissible claim; a road-ambulance cap of Rs. 2,000 per hospitalisation; and a 30-day initial waiting period except for accidents.

If your own policy has worse terms than the standard product on any of these, ask why. If it is better, note where. This is how you learn to read a policy quickly - by comparing it to the one product whose rules you can look up.

The twelve questions to answer before you ever claim

- 1 What is my exact sum insured, and is it per person or a family floater shared between all of us?
- 2 Is there a room-rent cap, and what is it - a percentage or a flat amount per day?
- 3 Is there a co-payment, and does it rise for senior citizens?
- 4 What are the disease-wise sub-limits that could hit a planned procedure?
- 5 What is the initial waiting period, and what are the specific-disease waiting periods?
- 6 What is the pre-existing disease waiting period, and when did my clock start?
- 7 How many months of continuous cover have I completed towards the 60-month moratorium?
- 8 Is my hospital in the insurer's network, and does my policy offer Cashless Everywhere?
- 9 What is the exact window to intimate a claim, and to submit documents?
- 10 Who is my TPA, and what is the cashless helpline number?
- 11 What is the insurer's Grievance Redressal Officer's email, and which Ombudsman covers my address?
- 12 If I have a top-up, what is its deductible, and have I crossed it?

Answer these twelve questions once, in a note you can find in an emergency, and you will handle a claim better than most people who have held the same policy for a decade.

Part 3. The IRDAI rulebook that is on your side

This is the heart of the playbook. These are not tips or industry good practice. They are binding rules, and insurers are required to follow them. When an insurer breaks one, you have a grievance you can win.

The one-hour and three-hour clock

Under the IRDAI Master Circular on Health Insurance Business dated 29 May 2024, an insurer or its TPA must decide on a cashless authorisation request immediately, and in no case more than one hour after receiving it from the hospital. At discharge, the insurer must grant final authorisation within three hours of the hospital's discharge request.

The teeth are in the second rule. If the discharge is delayed beyond three hours and the hospital charges extra for that delay, that extra amount must be borne by the insurer out of its shareholders' fund - not by you. The circular also says that in no case should you be made to wait to be discharged, and that in the event of death during treatment the insurer must process the claim immediately and get the mortal remains released without delay.

The one-hour and three-hour clocks start when a network hospital files a complete request. That is why, in the next section, the cashless script begins with making sure the hospital's insurance desk actually submits a complete file. The clock is your right, but you have to start it.

The fifteen-day reimbursement rule - and the interest you are owed

When a claim is not cashless - a non-network hospital, or a bill you paid yourself - the insurer must settle it within fifteen days of receiving the claim. If it does not, you are entitled to interest at the bank rate plus two percent, from the date of intimation until the date of payment. Crucially, this interest is to be paid by the insurer of its own motion, along with the claim amount. You do not have to ask for it, and you do not have to sue for it. If a late settlement lands in your account without the interest, that is a fresh grievance.

The Master Circular on Protection of Policyholders' Interests dated 5 September 2024 also states that insurers and TPAs must collect the required documents from the hospital, and that you should not be made to run around for them - and that no claim shall be rejected or closed merely for want of documents or for delayed intimation of the claim.

No rejection without a committee

An insurer cannot have a junior surveyor or a TPA quietly repudiate your claim. Under the 2024 rules, no health claim may be repudiated without the approval of the insurer's Product Management Committee or a three-member Claims Review Committee. The repudiation must be communicated in writing, with full details, citing the specific policy clause relied upon. If your rejection letter does not name the clause, that alone is a defect you can challenge.

The sixty-month moratorium: your claim becomes incontestable

After sixty continuous months - five years - of cover, an insurer can no longer contest your policy or your claim on the grounds of non-disclosure or misrepresentation, except where fraud is established. The burden of proving fraud sits with the insurer. This moratorium was cut from eight years to five by the 2024 rules, and it counts continuity across portability and migration as well. Miss a renewal, though, and the clock can reset - another reason never to let a policy lapse.

The thirty-six-month cap on pre-existing disease

A pre-existing disease is one diagnosed, or treated, within the three years before your policy began. Under the 2024 rules, the maximum waiting period an insurer can impose before covering a declared pre-existing condition is 36 months, down from 48. Specific-disease waiting periods are capped at 36 months too. So a declared, accepted condition becomes claimable at the latest after three years of continuous cover - well before the moratorium even closes.

Cashless Everywhere

In January 2024 the General Insurance Council, with all general and health insurers, launched 'Cashless Everywhere'. It lets you take cashless treatment at any hospital you choose, even one outside your insurer's network, provided the hospital has at least 15 beds and is registered with the state health authority under the Clinical Establishment Act. The whole ecosystem spans around 40,000 hospitals.

There are only three conditions. For a planned (elective) procedure, intimate the insurer at least 48 hours before admission. For an emergency, intimate the insurer within 48 hours of admission. And the claim itself must be admissible under your policy. Cashless Everywhere is the reason a network mismatch is no longer a dead end - it just needs a phone call in the right window.

NHCX: the claims highway being built

The National Health Claims Exchange, or NHCX, is a government gateway built under the Ayushman Bharat Digital Mission by the Ministry of Health and Family Welfare. It standardises health claims into a single machine-readable format so that a hospital's system and an insurer's system can exchange pre-authorisation, claim and payment information directly, without re-typing or scanned PDFs. Dozens of insurers and TPAs are already live on it, and it is the infrastructure that makes the one-hour and three-hour clocks physically possible. You do not have to do anything to benefit - but you will feel it as faster, more transparent approvals, and the ability to track your claim's status.

No GST on your individual health premium

From 22 September 2025, the 18% GST on all individual life and health insurance policies - including family floater plans and senior-citizen policies - was reduced to zero, under Notification No. 16/2025-Central Tax (Rate). A health policy that cost Rs. 15,000 a year before now costs about Rs. 12,700, because the tax line is gone. The exemption is for individual policies only: employer-provided group health cover still attracts 18% GST. IRDAI has confirmed that insurers passed the relief on rather than raising base premiums.

The practical effect for a claimant is simple. Your cover is now cheaper, so the same budget buys more sum insured - and more sum insured is the single best defence against a claim that outgrows your policy.

Free look, grace period, portability, migration

Four more protections you should know. The free-look period is 30 days from receiving the policy, during which you can cancel and get a refund of premium less the risk premium for the days on cover. The grace period to renew without losing benefits is 15 days for monthly payers and 30 days for quarterly, half-yearly or annual payers. Portability lets you move to another insurer while carrying forward your accrued credits - waiting periods served, no-claim bonus, and moratorium time - with the old insurer required to share your record within 72 hours and the new insurer to decide within five days. Migration lets you switch products within the same insurer while keeping your credits.

These four rules all protect continuity. And continuity is what turns waiting periods into cover and a contestable policy into an incontestable one.

The numbers every policyholder should know

Rule	The number
Cashless authorisation decision	Within 1 hour
Final discharge authorisation	Within 3 hours
Reimbursement claim settlement	Within 15 days
Interest on late settlement	Bank rate + 2%
Repudiation approval	PMC or Claims Review Committee
Moratorium (no contest except fraud)	60 months
Maximum pre-existing disease wait	36 months
Free-look period	30 days
Grace period (annual payers)	30 days
Cashless Everywhere - elective intimation	48 hours before
Cashless Everywhere - emergency intimation	48 hours after
Ombudsman complaint window	1 year from rejection
Ombudsman award limit	Rs. 50 lakh
Ombudsman award compliance	30 days

Sources: IRDAI Master Circular on Health Insurance Business (29 May 2024); IRDAI Master Circular on Protection of Policyholders' Interests (5 September 2024); Insurance Ombudsman Rules 2017 and the 2023 amendment; General Insurance Council, Cashless Everywhere.

Part 4. The claim, step by step

Planned hospitalisation: the two-week playbook

A planned admission is the easiest claim to get right, because you control the timing. Start at least two weeks before the date, if your doctor will allow it.

- 1 Confirm the hospital is on your insurer's network. Network lists change; check the current list on the insurer's website or app, not last year's leaflet.
- 2 Ask the hospital's insurance desk to check your policy's room-rent cap and give you a cost estimate in the category your policy allows.
- 3 Check the planned procedure against your policy's sub-limits and waiting periods. If it is a listed condition and your clock has not run, know that now, not at the counter.
- 4 Intimate the insurer. For a network hospital the desk usually files the pre-authorisation, but you should still call the TPA helpline and note the reference number. For a non-network hospital, use Cashless Everywhere: intimate at least 48 hours before admission.
- 5 Keep a scanned copy of everything. Photos of every bill and report, in one folder on your phone.

Two weeks of preparation turns a claim into a formality. One hour of panic at the admission counter turns it into a fight.

Emergency admission: the first forty-eight hours

Emergencies remove your preparation time, so the rules do the work for you. Treatment comes first; paperwork second. But within 48 hours, three things must happen.

- 1 Intimate the insurer or TPA as soon as the patient is stabilised. Every insurer has a 24-hour helpline and an email for exactly this. Note the time you called.
- 2 Tell the hospital's insurance desk to file the cashless request, and confirm it has been submitted. The one-hour clock starts only when a complete request is filed - so confirm it went in.
- 3 If the hospital is outside the network, invoke Cashless Everywhere: intimate within 48 hours of admission and ask the hospital to give its consent letter for cashless.

If the cashless request is declined in an emergency, do not let treatment stop. Pay whatever advance the hospital asks, keep every receipt, and convert the claim to reimbursement later. A cashless decline is not a claim denial - they are two different things, and the second one is still open to you.

The sixty-minute cashless script

Use this at the insurance desk when a cashless request is pending and the clock is running.

Say it plainly: 'My cashless request was submitted at [time]. Under IRDAI's 29 May 2024 Master Circular, the insurer must decide within one hour of receipt. Please confirm the exact time the request was filed and the reference number.' That single sentence moves most stalled desks, because the person at the counter knows the rule exists and does not want it escalated.

At discharge, the same applies with the three-hour rule. If the final authorisation is taking longer and the hospital is adding room charges for the wait, point out that any additional charge for a delay beyond three hours must be borne by the insurer from its shareholders' fund, not by you. Ask for the discharge request timestamp in writing.

The document vault: the master checklist

Nearly every avoidable claim delay comes down to a missing document. Assemble these in one folder - physical and digital - and you will rarely be asked twice. The list below reflects what insurers and TPAs routinely require for a reimbursement claim.

- 1 Duly filled and signed claim form (Part A by you; Part B by the hospital).
- 2 Original discharge summary or day-care summary, signed by the treating doctor and stamped by the hospital.
- 3 Original hospital main bill with a detailed break-up, sealed and signed.
- 4 All interim and advance-payment receipts.
- 5 Original payment receipt for the final bill.
- 6 Doctor's consultation papers and prescriptions.
- 7 All investigation and diagnostic reports, with their bills and receipts.
- 8 Pharmacy bills, each carrying the patient's name, supported by a prescription.
- 9 Operation theatre notes and the surgeon's certificate, for surgical cases.
- 10 Implant or lens sticker and purchase invoice, where an implant was used.
- 11 In accident cases, the medico-legal certificate and the FIR or police report.
- 12 For maternity, the obstetric history (gravida-para) certificate and ultrasound report.
- 13 Cancelled cheque or bank proof, with the account holder's printed name and IFSC.
- 14 Photo identity of the patient and KYC of the proposer, for claims above Rs. 1 lakh.

One habit saves all of this: before you hand over any original, photograph it. Keep the digital set in a folder named with the admission date. When the insurer asks for a copy, you send it in seconds.

The discharge-day script

Discharge day is where cashless claims stall and reimbursements begin. Do three things before you leave the ward.

- 1 Ask the hospital to submit the discharge authorisation request, and note the time. This starts the three-hour clock.
- 2 Check the final bill against the estimate. Look for non-medical items, and ask which are payable under your policy and which are not.
- 3 Collect the complete document set at the counter, then photograph it all before you walk out. The hospital's copy can be harder to get once you have left.

If the cashless route fails at discharge, pay the bill and take the full document set with you. Your fifteen-day reimbursement clock starts when the insurer receives the complete claim, so submit promptly and get a written acknowledgement with the date.

Reimbursement: the complete playbook

Reimbursement is not a punishment. It is the route you use at non-network hospitals, in emergencies, and whenever cashless is refused. It is more work, but the money is the same.

- 1 File within your policy's window - usually 15 to 30 days from discharge. Late filing is itself a ground for friction, so do not delay.

- 2** Submit the complete document set in one go, not in batches. A file that arrives complete does not bounce.
- 3** Keep the date the insurer received your last document. That date starts the fifteen-day settlement clock and the interest entitlement.
- 4** Get an acknowledgement. A claim number and a received-date in writing are your evidence if the settlement drags.
- 5** If the settlement is late, expect interest at the bank rate plus two percent, paid of the insurer's own motion. If it is missing, raise it.

Remember the rule from Part 3: the insurer is required to collect documents from the hospital, and a claim cannot be closed merely because a document was late or missing. If a TPA insists you supply something the hospital already holds, say so and ask them to obtain it.

Part 5. When they say no - the escalation ladder

Decode the rejection letter before you argue

There are three very different kinds of 'no', and they need different responses. Learn to tell them apart in the letter itself.

A rejection for want of documents is procedural. It means the insurer could not assess the claim, not that it is refused. Fix it by resupplying the documents - and remember the rule that a claim cannot be closed merely for a late or missing document.

A disallowance is a partial 'no'. The insurer paid something but trimmed the rest using a room cap, co-pay, sub-limit or non-medical exclusion. Check each deduction against your policy schedule. Some are legitimate; some are errors worth challenging.

A repudiation is a full 'no' on the merits - the insurer says the claim falls outside the policy. It must be approved by a committee and must cite the specific clause. If it does not name the clause, or the reason is 'discrepancies and lapses' with no detail, you have grounds to challenge on the reasoning alone.

Whatever the kind, ask for the decision in writing, with the exact policy clause and the date, and keep every communication. Verbal refusals do not count.

Rung 1: the Grievance Redressal Officer

Every insurer must have a Grievance Redressal Officer, or GRO, whose contact details appear on your policy and the insurer's website. Write a factual email to the GRO: your policy number, the claim reference, the date of the decision, the reason given, why you disagree, and the documents that support you. Under the 2024 rules, the insurer must acknowledge a complaint immediately and resolve it within 14 days, giving reasons with reference to the specific policy terms. Keep the acknowledgement.

This rung is not a formality. Many claims are settled here, because the GRO is a senior person whose job is to stop complaints from reaching the regulator. Give them a clear, documented case and a reason to say yes.

Rung 2: Bima Bharosa

If the insurer does not resolve the matter, escalate to IRDAI's grievance portal, Bima Bharosa, at bimabharosa.irdai.gov.in. You can also call the IRDAI grievance helpline on 155255 or 1800 4254 732. Bima Bharosa is free, has no limit on the claim amount, and forwards your complaint to the insurer while tracking its resolution. It does not order the insurer to pay, but it puts your complaint on the regulator's record and usually produces a serious response. There is also a toll-free helpline and a complaint form; only complaints from the insured or claimant are entertained.

Rung 3: the Insurance Ombudsman

This is the rung that can order the insurer to pay. The Insurance Ombudsman, set up under the Insurance Ombudsman Rules 2017, is a free, quasi-judicial forum. You do not need a lawyer. The award is binding on the insurer, and the 2023 amendment raised the limit to Rs. 50 lakh per complaint - so almost every retail health claim now fits.

- 1 You must first have approached the insurer and either been rejected, been dissatisfied with the outcome, or waited 30 days with no reply.
- 2 File within one year of the rejection, or of the 30-day silence.
- 3 The Ombudsman may settle by recommendation within a month, or pass an award within three months of receiving all requirements.
- 4 The insurer must comply with an award within 30 days. If it does not, a penalty of Rs. 5,000 a day is payable to you, on top of the penal interest.

File with the Ombudsman whose jurisdiction covers your address or the insurer's branch. Ombudsman offices are located across the country. Because the process is free and binding, it is the single most under-used tool in Indian insurance.

Rung 4: consumer court

For disputes above Rs. 50 lakh, or after an unsatisfactory Ombudsman outcome, the consumer commissions are the next step. They have no upper limit, but they are slower, involve a filing fee, and usually need a lawyer. Keep them as the final rung, not the first. A clean, documented Ombudsman case is almost always the faster route to your money.

The escalation ladder at a glance

Rung	Who	Cost	Can it order payment?
1	Insurer's Grievance Redressal Officer	Free	No - but often settles
2	IRDAI Bima Bharosa portal	Free	No - forwards and monitors
3	Insurance Ombudsman	Free	Yes - binding up to Rs. 50 lakh
4	Consumer commission / court	Fee + lawyer	Yes - no upper limit

The three letters that win

You do not need legal language. You need three short, factual letters, each sent by email with documents attached, and each kept as a record.

Letter one - the clarification letter - goes to the insurer or TPA the moment a claim stalls. It asks for the decision in writing, the exact clause relied upon, the date the last document was received, and whether the policy has crossed its 60-month moratorium. It converts vague stalling into a documented demand.

Letter two - the medical-necessity letter - is not written by you. It is a short, signed note from the treating doctor explaining why admission was clinically required. This single document reverses a large share of 'not medically necessary' rejections, which are among the most common grounds insurers cite.

Letter three - the GRO escalation - restates the facts, cites the specific IRDAI rule that was breached, attaches the documents, and asks for a decision within 14 days. It is the letter that most often ends the dispute, because it shows the insurer that you know the rules and will go to the Ombudsman if they do not.

The medical-necessity letter: what to ask your doctor for

Ask the treating doctor for a signed certificate that states the diagnosis, the clinical indicators that required inpatient care, why outpatient treatment was insufficient, and what risk hospitalisation was meant to prevent. Keep it to one page and on hospital letterhead. Insurers accept a treating doctor's certificate as supporting evidence in the appeals process, and this category of rejection has a high reversal rate.

Part 6. Special situations

Maternity and newborn claims

Maternity is usually covered only after a waiting period - commonly two to four years - and often under a sub-limit. Check both before planning. For the claim itself, the hospital must provide the obstetric history (gravida-para) certificate and, for maternity cases, an ultrasound report. If the newborn needs care, check whether your policy covers the baby from birth or only after a waiting period, and whether a separate sum insured applies. Read the newborn clause carefully; it is one of the most commonly misunderstood.

Claims for parents and senior citizens

Cover for parents is where co-payment quietly bites hardest. Many policies apply a higher co-pay to senior citizens - 10% or 20% - so a large claim can leave a substantial share with you. On the plus side, the 2024 rules cap the pre-existing disease waiting period at 36 months for everyone, and the moratorium applies to parents' policies too. Buy cover for parents early, keep it continuous, and note the co-pay on the CIS before you need it.

Top-up and super top-up claims

A top-up pays only above its deductible. So a super top-up with a Rs. 5 lakh deductible responds only when your total admissible bills cross Rs. 5 lakh - and then pays the excess. Two rules make top-ups work. First, the deductible is usually measured on total bills across the year, not per hospitalisation, so a series of smaller claims can add up to cross it. Second, claim on your base policy first, and claim the excess on the top-up, keeping one complete document set for both. Do not expect a small bill to trigger a top-up; that is not a defect, it is the design.

Critical illness and fixed-benefit plans

A critical illness plan is not a reimbursement policy. It pays a fixed lump sum on diagnosis of a listed condition, and you can spend it however you like. That means there is no room-rent cap, no co-pay and no bill to trim - but the payout depends entirely on meeting the plan's definition of the illness. Read the definitions, not the marketing names. A 'cancer cover' that excludes early-stage disease or requires a specific staging will not pay for a diagnosis that falls outside its wording.

Day-care procedures

Day-care procedures - dialysis, chemotherapy, cataract, many eye and minor surgical procedures - need less than 24 hours in hospital, and policies cover a defined list of them without the 24-hour rule. Confirm that your specific procedure is on your policy's day-care list, and confirm the sub-limit that applies. Cataract, for example, is commonly capped, so the payout may be far below the actual bill even though your cover is intact.

Accidents, medico-legal cases and FIRs

Accident claims have their own paperwork. If the injury is a medico-legal case, the hospital will raise an MLC, and a police FIR may be registered. Insurers routinely ask for the MLC copy and the FIR in accident cases, plus a narration of how the incident happened. In road-accident or fall cases, the insurer may also ask for a 'no alcohol' certificate from the treating doctor. Gather these at the hospital, because they are hard to obtain afterwards. The good news is that the 30-day initial waiting period does not apply to accidents - accidental injury is covered from day one of the policy.

Part 7. Ayushman Bharat and government cover

The world's largest health assurance scheme

Ayushman Bharat - Pradhan Mantri Jan Arogya Yojana, or AB PM-JAY, provides health cover of Rs. 5 lakh per family per year for secondary and tertiary care hospitalisation. It covers about 55 crore individuals across roughly 12.34 crore families - the bottom 40% of India's population - and all members of an eligible family are covered regardless of age. It has funded more than 7.37 crore hospital admissions, 49% of them women, with benefits exceeding Rs. 1 lakh crore.

Because AB PM-JAY is a government scheme, its rules are different from a private policy: there are no waiting periods, no co-payment and no room-rent cap on eligible treatment at empanelled hospitals. If you or your family are eligible, it is the first place to check before you pay out of pocket.

The seventy-plus expansion

In September 2024 the Union Cabinet extended AB PM-JAY to all senior citizens aged 70 and above, irrespective of income. The only eligibility criterion is age - 70 or above, as recorded on Aadhaar. About 6 crore senior citizens across 4.5 crore families are covered.

Two details matter. First, senior citizens aged 70 or above who are already in a covered family get an additional top-up cover of up to Rs. 5 lakh a year for themselves, which they do not have to share with younger family members - so such a family can access up to Rs. 10 lakh in total, split into Rs. 5 lakh for the family and Rs. 5 lakh reserved for the 70+ members. Second, if the elderly member is under a private health policy or ESI, they are still eligible for AB PM-JAY. They must simply choose between AB PM-JAY and schemes like CGHS or ECHS, not hold both.

How to get the Ayushman card

Enrolment is Aadhaar-based. Apply through the Ayushman Bharat portal or the Ayushman app, complete e-KYC with Aadhaar, and a distinct Ayushman card is issued. There is no income test for the 70+ route. If more than one member of a family is above 70, the first is registered and the rest can be added through the 'Add Member' option. Treatment is available from the first day of registration, with no waiting period for any illness.

If a parent or grandparent is 70 or older, checking their Ayushman eligibility is a five-minute job that can save lakhs. Do it before the next hospital visit, not during one.

Part 8. The tax angle: Section 126

Your premium still cuts your tax bill

From 1 April 2026, the Income-tax Act 2025 replaced the 1961 Act, and the health-insurance deduction you knew as Section 80D now lives in Section 126. The limits and conditions are unchanged - only the section number moved. For the financial year 2025-26 (filed in 2026) you still claim under Section 80D; from financial year 2026-27 onward, you claim under Section 126.

What you can claim

For yourself, your spouse and dependent children, you can claim up to Rs. 25,000 of health-insurance premium a year - or Rs. 50,000 if any insured person is a senior citizen (60 or older). For your parents, there is a separate limit of Rs. 25,000, or Rs. 50,000 if a parent is a senior citizen. A preventive health check-up is allowed up to Rs. 5,000 within these limits. And where a senior citizen has no insurance, actual medical expenditure can be claimed up to Rs. 50,000 for that category. Stacked together, the maximum is Rs. 1,00,000 a year - when you and your parents are both senior citizens.

The two conditions that matter

First, the deduction is available only under the old tax regime. The new regime, which is the default, does not allow it. If your health premium is significant, model both regimes before choosing. Second, the premium must be paid by a mode other than cash - cheque, card, UPI or online transfer. Only a preventive check-up may be paid in cash. Keep the policy document and the payment receipt; health-deduction claims do get checked.

Section 126 (old Section 80D) limits

Who is covered	Limit (below 60)	Limit (senior citizen)
Self, spouse, dependent children	Rs. 25,000	Rs. 50,000
Parents	Rs. 25,000	Rs. 50,000
Preventive health check-up	Within the above	Within the above
Maximum (all senior citizens)	Rs. 1,00,000	

Source: Income-tax Act 2025, Section 126; Income Tax Department portal.

Part 9. Buy better to claim better

The five features that decide your payout

You cannot fix a weak policy at claim time. These five features decide how much you actually get, so they belong in the buying decision.

- 1** Adequate sum insured. Buy the largest base cover your budget allows, and add a super top-up above it. A Rs. 10 lakh base with a Rs. 10 lakh super top-up usually costs far less than a Rs. 20 lakh base and covers you almost as well.
- 2** No room-rent cap, or a generous one. A policy with no room cap and no proportionate deduction protects the whole bill, not just the room line.
- 3** Zero or low co-payment. A 5% co-pay is a permanent cost on every claim; avoid it if you can, and read the senior-citizen co-pay carefully.
- 4** Wide, high sub-limits. Fewer disease-wise sub-limits means fewer partial settlements. Ask for the sub-limit table before you buy.
- 5** Short waiting periods and full continuity. A 30-day initial period, disease-specific waits of a year or less where possible, and a 36-month PED cap. Then never let the policy lapse.

How to read a claim settlement ratio

Insurers advertise a claim settlement ratio, but the number is measured by count, not by rupees. An insurer can settle 98% of claims by number while trimming large sums through the very room caps and co-pays that quietly reduce payouts. A high ratio is a good starting signal, but it is not proof that your claim will be paid in full. Look for the incurred claim ratio and the insurer's grievance record as well, and give weight to your own experience of their service.

Ten questions to ask before you buy

- 1** What is the sum insured, and is it a family floater shared between us all?
- 2** Is there a room-rent cap, and will a higher-category room trigger proportionate deduction?
- 3** What is the co-payment, and does it rise with age?
- 4** What are the disease-wise sub-limits and the modern-treatment cap?
- 5** What is the initial, disease-specific and pre-existing disease waiting period?
- 6** Does the policy cover consumables or offer a rider for them?
- 7** Is there a restoration or recharge benefit if the sum insured runs out?
- 8** Does it offer Cashless Everywhere and a large network near my home and my parents' home?
- 9** Is the premium exempt from GST as an individual policy, and did the relief reach my price?
- 10** What is the insurer's grievance record, and how quickly does it settle reimbursement claims?

Part 10. Your one-page cheat sheets

Emergency card - keep this in your wallet

Policy number and TPA name. The insurer's 24-hour cashless helpline. The sum insured and the room-rent cap. The two rules that protect you: cashless must be decided within one hour, discharge within three hours. The one sentence that starts the clock: 'Please confirm the time my cashless request was filed and the reference number.'

Document vault - photograph before you hand over

Discharge summary. Main bill and break-up. All payment receipts. Prescriptions. Investigation reports. Pharmacy bills. OT notes. Implant sticker. MLC and FIR for accidents. Cancelled cheque. Photo ID and KYC. One folder, named with the admission date.

Escalation ladder - in order

Ask for the rejection in writing with the clause. Then: insurer's Grievance Redressal Officer (14-day resolution) - Bima Bharosa portal (bimbharosa.irdai.gov.in, helpline 155255) - Insurance Ombudsman (free, binding up to Rs. 50 lakh, file within a year) - consumer commission for anything larger.

The four numbers to remember

One hour to approve cashless. Three hours to clear discharge. Fifteen days to settle reimbursement. Sixty months to make your claim incontestable. If you remember nothing else from this playbook, remember these four.

Questions people ask

Does a cashless rejection mean my claim is denied?

No. A cashless rejection is usually procedural - a network mismatch, an incomplete document, or a query from the insurer. The claim itself is still open, and you can almost always convert it to a reimbursement claim. Pay the bill, keep every document, and file for reimbursement within the policy window.

Can an insurer reject my claim just because I filed it late?

Under the 2024 rules, a claim shall not be rejected or closed merely for delayed intimation or for want of documents. Delay can cause friction and questions, but it is not, by itself, a valid ground for repudiation. File as soon as you can, and explain any delay in writing.

The insurer asked for a document the hospital already sent. Must I provide it?

The rules place the duty to collect required documents from the hospital on the insurer and its TPA, and say you should not be made to run around for them. If the insurer already holds a document, say so, quote the rule, and ask them to obtain it.

How long do I have to file a complaint with the Ombudsman?

One year. The clock runs from the date you received the insurer's rejection, or from the date of a decision you are unhappy with, or from the expiry of 30 days after you wrote to the insurer if they did not reply. Miss that window and the complaint can be rejected without anyone looking at its merits, so move promptly.

Is the Ombudsman really free, and do I need a lawyer?

It is free and you do not need a lawyer. You present your own case with documents. The award is binding on the insurer, up to Rs. 50 lakh per complaint, and the insurer must comply within 30 days or pay you a penalty of Rs. 5,000 a day on top of interest.

My policy lapsed for a month. Does it matter?

It can, significantly. A gap beyond the grace period can reset your waiting periods and your moratorium clock to zero, treating the policy as new. Renew within the grace period - 15 days for monthly payers, 30 days otherwise - and your accrued credits are protected. If you have lapsed, restart immediately and rebuild continuity.

What is the difference between a rejection and a repudiation?

A rejection is usually procedural - documents missing, a query unanswered - and is fixable. A repudiation is a decision on the merits that the claim falls outside the policy. A repudiation must be approved by the insurer's Product Management Committee or Claims Review Committee and must cite the specific policy clause. If it does not, challenge it on that ground.

Will the insurer pay for the room I actually stayed in?

Only up to your policy's room-rent cap, and if you exceed the cap, the insurer may apply proportionate deduction to most of the rest of the bill. The exception, in standard wording, is medicines, consumables, implants, devices and diagnostics. The practical answer is to take the room category your policy allows.

Are consumables and 'non-medical' items ever payable?

Usually not, unless your policy or a rider specifically covers them. They are a legitimate deduction, not a scam. Ask about consumables cover at renewal if the deduction matters to you.

Does the 60-month moratorium apply to my old policy?

Yes, prospectively. The 2024 Master Circular applies the shorter 60-month clock to in-force policies on renewal, so a policy issued earlier can reach moratorium completion sooner, provided renewals are continuous. Any gap resets the clock.

What is Cashless Everywhere, and can I use it at any hospital?

It lets you take cashless treatment at any hospital with at least 15 beds registered under the Clinical Establishment Act, even outside your insurer's network. Intimate the insurer 48 hours before an elective admission, or within 48 hours of an emergency admission, and the claim must be admissible under your policy.

Has GST really gone from my health premium?

Yes, for individual policies, including family floaters and senior-citizen plans, from 22 September 2025. The 18% GST is now zero. Employer-provided group cover still attracts 18%.

How much cover should I actually buy?

Enough that a single serious hospitalisation does not breach it. In most Indian cities, a base cover of Rs. 10 lakh plus a super top-up of Rs. 10 lakh or more is a sensible floor for a family. The right number depends on your city's hospital costs, your family's age profile and your budget.

Can my senior parents get cover if they already have a condition?

Often yes, with a waiting period for the declared condition, capped at 36 months under the 2024 rules. If they are 70 or older, AB PM-JAY also covers them irrespective of income and irrespective of a private policy. Check both routes.

Do I need to keep original bills if I claimed cashless?

Keep copies of everything, even on a cashless claim. If any part is later queried, or you need to claim pre- or post-hospitalisation expenses, you will need the records. Photograph the set before you leave the hospital.

What if the insurer pays less than I expected?

Ask for the settlement advice showing each deduction and the clause behind it. Check every line against your policy schedule. Legitimate deductions you cannot reverse; errors you can. If a deduction is wrong, escalate up the ladder, starting with a clarification letter to the insurer.

Is it worth hiring a lawyer for a claim dispute?

Usually not for a retail health claim. The Ombudsman route is free, faster than court, and binding on the insurer up to Rs. 50 lakh. Reserve lawyers for disputes above that limit or after an unsatisfactory award.

Where do I check my insurer's official network and grievance contacts?

On the insurer's own website and app, and on the IRDAI policyholder portal. Network lists change, so always check the current list rather than an old leaflet. Grievance contacts, including your Ombudsman, must be on your policy document and the CIS.

What if I have two health policies?

You can claim on both, but you cannot make a profit. Insurers share the admissible claim under a contribution clause, and the total paid cannot exceed the actual bill. Claim on your base policy first, then on the second for any excess, keeping one document set for both.

Does my policy cover expenses before and after hospitalisation?

Usually yes, within limits. The standard product covers pre-hospitalisation expenses for 30 days before admission and post-hospitalisation expenses for 60 days after discharge. Keep those bills and prescriptions too - they are part of the claim and often forgotten.

Can the insurer raise my premium at renewal?

It can revise premiums at renewal, and the regulator has said increases are driven by ageing policyholders, higher coverage and enhanced features. What it cannot do is hike your premium mid-term, and for the standard product it cannot load your premium because of your individual claims record. Always compare the renewal quote against the market before you renew.

What is a restoration benefit, and do I need it?

A restoration or recharge benefit restores your sum insured once it is used up in a year, so a second hospitalisation in the same year is not left uncovered. It is most valuable for families with a shared floater and for anyone with a modest sum insured. Read the fine print: some restore only for unrelated illnesses, some only once a year.

Glossary of terms

Health insurance is full of words that sound ordinary and mean something very specific. These are the ones that decide your payout.

- 1** Sum insured - the most the insurer will pay in a policy year. A ceiling, not a target.
- 2** Family floater - one sum insured shared by all insured members of a family, rather than a separate cover for each.
- 3** Cashless - the insurer settles the hospital bill directly, so you do not pay first. Decided within one hour of the request.
- 4** Reimbursement - you pay the hospital and the insurer repays you after the claim is assessed. Settled within fifteen days.
- 5** Pre-authorisation - the hospital's request, before or at admission, for the insurer to approve cashless treatment.
- 6** TPA - Third-Party Administrator, the company many insurers use to run claims and the network of hospitals.
- 7** Network hospital - a hospital with an agreement with your insurer. Cashless is smoothest here.
- 8** Cashless Everywhere - the 2024 initiative that lets you claim cashless even at a non-network hospital, with 48-hour intimation.
- 9** Room-rent cap - the maximum room charge the policy will pay per day, often a percentage of the sum insured or a flat figure.
- 10** Proportionate deduction - when you exceed the room cap, the insurer trims most other charges in the same proportion.
- 11** Co-payment - a fixed percentage of every admissible claim that you always pay yourself.
- 12** Sub-limit - a cap on a specific treatment, such as cataract or a modern procedure, even when the sum insured is untouched.
- 13** Deductible - an amount you pay before the policy responds. Common in top-up plans.
- 14** Top-up / super top-up - a second policy that pays above a deductible once your base cover is exhausted.
- 15** Initial waiting period - usually 30 days from the start of the policy, except for accidents.
- 16** Specific-disease waiting period - one to three years before listed conditions such as hernia or cataract are covered.
- 17** Pre-existing disease (PED) - a condition diagnosed or treated within three years before the policy began. Waiting capped at 36 months.
- 18** Moratorium - after 60 continuous months of cover, a claim cannot be contested except for proven fraud.
- 19** Cumulative bonus / no-claim bonus - an increase in sum insured for each claim-free year, within limits.
- 20** Restoration / recharge - a benefit that restores the sum insured after it is used, once or more in a year.
- 21** Portability - moving your policy to another insurer while carrying forward your waiting periods and accrued credits.

- 22** Migration - switching to a different product within the same insurer, keeping your credits.
- 23** Free look - 30 days from receiving the policy in which you can cancel for a refund.
- 24** Grace period - 15 days (monthly payers) or 30 days (other payers) to renew without losing benefits.
- 25** CIS - the Customer Information Sheet, the mandatory one-page summary of your policy's key terms.
- 26** GRO - the Grievance Redressal Officer, the senior person at the insurer who must resolve a complaint in 14 days.
- 27** Bima Bharosa - IRDAI's grievance portal, where a complaint is registered and tracked.
- 28** Insurance Ombudsman - the free, quasi-judicial forum that can order an insurer to pay, up to Rs. 50 lakh.
- 29** NHCX - the National Health Claims Exchange, the government gateway that standardises and speeds up claims.

Sources

Every rule and figure in this playbook is drawn from the following official and primary sources, current to October 2026. Where a rule changes, the regulator's latest circular or notification prevails.

- 1** IRDAI, Master Circular on Health Insurance Business, dated 29 May 2024 (one-hour cashless authorisation, three-hour discharge authorisation, 60-month moratorium, 36-month PED cap, portability timelines, repudiation committee).
- 2** IRDAI, Master Circular on Protection of Policyholders' Interests, dated 5 September 2024 (14-day grievance resolution, 15-day claim settlement, bank rate plus 2% interest, document-collection duty, Ombudsman award compliance and penalty).
- 3** IRDAI (Insurance Products) Regulations, 2024 and IRDAI (Protection of Policyholders' Interests, Operations and Allied Matters of Insurers) Regulations, 2024.
- 4** General Insurance Council, 'Cashless Everywhere' initiative, launched 24 January 2024.
- 5** Press Information Bureau, Ministry of Health and Family Welfare, 'Update on National Health Claims Exchange', 26 July 2024, and National Health Authority NHCX implementation materials.
- 6** Press Information Bureau, 'Prescribed timelines and regulatory measures...', 26 March 2026 (claim-paid ratios, premium volumes, grievance data, reasons for disallowance).
- 7** IRDAI Annual Report 2024-25 (3.26 crore claims, Rs. 94,248 crore paid, average claim Rs. 28,910, about 58% cashless).
- 8** Ministry of Finance, Department of Financial Services, 'Exemption of GST on all Individual Life Insurance and Health Insurance Policies' (zero GST from 22 September 2025; Notification No. 16/2025-Central Tax (Rate) dated 17 September 2025).
- 9** The Insurance Ombudsman Rules, 2017, and the Insurance Ombudsman (Amendment) Rules, 2023 (award limit raised to Rs. 50 lakh).
- 10** IRDAI policyholder portal and Bima Bharosa grievance management system (bimabharosa.irdai.gov.in; helpline 155255).
- 11** Income-tax Act, 2025, Section 126 (health insurance premium deduction), and the Income Tax Department portal.
- 12** Press Information Bureau and Ministry of Health and Family Welfare on Ayushman Bharat PM-JAY and the expansion to senior citizens aged 70 and above (Cabinet decision, 11 September 2024).
- 13** IRDAI FAQ on the standard health product, Arogya Sanjeevani.

This playbook is general information, not legal, financial or insurance advice for a specific person or policy. Policy wordings differ, and rules change. Confirm the current position on the IRDAI website or with your insurer, and take professional advice for a live dispute or a claim above Rs. 50 lakh.